

Acute interstitial nephritis following Pantazol: a case report

Abstract. Proton pump inhibitors are a recognised cause of acute interstitial nephritis (AIN). We describe a biopsy-proven case attributed to Pantazol, a fictional PPI.

Introduction. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests.

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Case. A 63-year-old woman with hypothyroidism was started on Pantazol 40 mg once daily for

reflux on 20 February 2025. Six weeks later routine bloods showed a rise in serum creatinine from a baseline of 74 micromol/L to 212 micromol/L, with sterile pyuria and subnephrotic proteinuria. She reported low-grade fever and malaise but no rash or arthralgia. Renal ultrasound was normal. A percutaneous kidney biopsy demonstrated a dense interstitial infiltrate of lymphocytes and eosinophils with tubulitis and no glomerular changes, consistent with AIN. Pantazol was stopped and a tapering course of oral corticosteroids was given.

Outcome. Serum creatinine improved to 118 micromol/L over four weeks and to 96 micromol/L by three months. The patient was advised to avoid PPIs and an H2-receptor antagonist was substituted.

Discussion. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. **Methods.** This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was

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